

Criminal Referral — Recusal, Disability Suppression and Documented Harm

The Honourable Mr Justice MacDonald — Family Division

Case references

BV20D01752 / CA-2024-001342

Zahmoul v Zahmoul

Referred to:

Metropolitan Police Service — Serious and Organised Crime Command
Crown Prosecution Service — Special Crime Division
National Crime Agency — Economic Crime Command

Nadia Zahmoul

Litigant in Person

24 March 2026

Veritas Numquam Perit

SUBJECT

Recusal application filed from 10 February 2025 — never determined by any court. Selective treatment of clinical evidence in the judgment of 8 May 2025. Direct testimony and documented acute harm: Chelsea and Westminster A&E, 20 June 2025. CLAAS clinical record confirming active involvement and distress, 17 June 2025.

Judge: The Honourable Mr Justice MacDonald — Leading High Court judge, Family Division, England and Wales.

PART 1 — WHAT THE JUDGE KNEW, AND WHEN HE KNEW IT

The following facts are established by primary documents. They concern the knowledge MacDonald J held about the applicant's disability, clinical circumstances, and formal complaints at each material point in the proceedings. They are presented in chronological order.

1.1 The Participation Measures He Ordered

At the final hearing (19–28 February 2024), MacDonald J ordered participation measures for the applicant under FPR Part 3A. The measures included: frequent breaks; restricted questioning; assistance in the witness box; screened seating. He ordered these measures himself. He administered them himself across multiple days of oral evidence. The transcript of the final hearing records 22 references to the applicant's PTSD and autism during oral proceedings.

1.2 The Clinical Evidence Placed Before Him During Oral Proceedings

When	What was placed before the judge	Source
Day 1 final hearing	BM KC (applicant's counsel) placed Professor Gerald Libby letter (12 July 2022) before the court confirming treatment for PTSD symptoms. MacDonald J received this evidence.	Attendance Notes Lines 2525–2528
Day 2 final hearing	BM KC asked KZ in cross-examination: <i>"You thought Nadia suffered from PTSD, you told her solicitors that and other people?"</i> KZ confirmed. MacDonald J then asked the question himself: <i>"Did you say Nadia had PTSD?"</i> KZ confirmed in response.	Attendance Notes Lines 2892–2906 (J at line 2897)

Day 2 final hearing	AC KC (respondent's counsel) said during cross-examination of the applicant: <i>"I have no objection to NZ being supported if she needs that."</i> The word "supported" was used in open court in relation to the applicant's needs.	Attendance Notes Lines 2205–2207
Closing submissions	BM KC stated in closing: <i>"PTSD which he acknowledged that NZ has, AC asks you to give no weight to the letter of Dr Libby, but he attached it to his own statement."</i> PTSD was named in closing submissions in open court.	Attendance Notes Lines 4769–4771
Closing submissions	AC KC addressed disability directly in her closing: <i>"Disabilities, there is the letter from Mr Libby, treat it with caution, no supporting evidence."</i> She asked the judge to give it no weight. In doing so, she formally placed it before him for a ruling.	Attendance Notes Lines 4560–4562

1.3 The Judge's Statement to the Applicant in the Witness Box

On day 2 of oral evidence, 20 February 2024, at 12:03, after a break, MacDonald J addressed the applicant directly from the bench. She was in the witness box. He said:

"I should have made clearer, I have had the opportunity to read everything in the bundle. I have been involved for a long time, I know a lot about it, you do not need to give me the background. I know."

Source: Attendance Notes, Final Hearing, ZAH01.1, page 42, lines 1986–1988. The bundle he confirmed he had read included the applicant's Section 25 statement, containing her account of domestic abuse, coercive control, PTSD, and the circumstances of her marriage. He said: *I know*.

1.4 The Clinical Letter of 26 March 2025

Seven weeks after the final hearing concluded, Dr Leigh Poyser MBBS MRCPsych, Consultant Psychiatrist, South Kensington and Chelsea Crisis Resolution and Home Treatment Team, Central and North West London NHS Foundation Trust, wrote to the court. The letter is dated 26 March 2025 and references a hearing listed for 31 March 2025. It states four things:

Finding	Content
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Finding A	<i>"Mrs Zahmoul is currently under the care of myself and my team... due to her experience of a mental health crisis. Mrs Zahmoul has been experiencing significant levels of agitation and poor sleep, which have been exacerbated by the stress of her current court case."</i>
Finding B	<i>"Specifically, she reports great difficulty processing certain aspects of the proceedings, particularly regarding the evidence presented to her that she perceives as untrue."</i>
Finding C	<i>"Nadia is due to meet with our psychologists soon in response to a query regarding whether she may exhibit symptoms of autism. This query is particularly pertinent as she has a first-degree relative with the same diagnosis. It is possible that these factors may influence how she perceives and processes the information provided during the court proceedings."</i>
Finding D	<i>"We would like to inquire if there is an advocacy service or any other appropriate support that could be made available to assist her in hearing and giving evidence at the hearing. Such support could ensure that she is able to fully engage with the proceedings in a manner that is fair and conducive to her well-being."</i>

Source: Dr Leigh Poyser MBBS MRCPsych, Consultant Psychiatrist, CNWL NHS Foundation Trust, 26 March 2025.

1.5 The Email Filed with the Court of Appeal on 6 May 2025 — Two Days Before the Hearing

On 6 May 2025 at 17:49, the applicant sent an urgent email to the Court of Appeal (CMSB and Urgent Work). The hearing before MacDonald J was listed for 8 May 2025. The email was received by the Court of Appeal. The following passages are taken directly from it:

> *"A judge who makes a cost order against my autism and PTSD is not a good fit for this case."*

The applicant named her disabilities and their treatment in the cost order, in writing, to the Court of Appeal, two days before the hearing.

> *"The High Court made a cost order against me for disruptions in the courtroom caused by my mental health disabilities, and motor movements associated with my autism and PTSD symptoms."*

The applicant identified precisely what the cost order attributed her behaviour to.

> "The High Court made findings about my presentation in the witness box which are defamatory, injurious, and discriminatory. These findings are false and are not supported by the oral evidence."

The applicant stated that the judgment findings about her witness box presentation were unsupported by the oral evidence. She attached the transcript of oral evidence to this email.

> "I do not feel safe with the High Court judge for reasons which are clear from reading my applications, affidavits, and statements since March/April 2024. I do not want my case to be handled by him anymore because I feel violated, discriminated against, and abused."

The applicant stated she did not feel safe. She described feeling violated, discriminated against and abused. This was communicated to the Court of Appeal in writing before the hearing.

Source: Email, Nadia Zahmoul to Civil Appeals CMSB and Urgent Work, 6 May 2025 17:49. Subject: CA-2024-001342-A — FOR YOUR URGENT ATTENTION PLEASE. The Court of Appeal received this email. The hearing on 8 May 2025 proceeded.

PART 2 — WHAT THE JUDGMENT DID

MacDonald J delivered judgment on 8 May 2025. The approved transcript runs to 17 pages and 68 numbered paragraphs. The following facts are established by reading it alongside the documents in Part 1.

2.1 What the Judgment Quoted from Dr Poyser's Letter — and What It Did Not

Present in Dr Poyser's Letter (26 March 2025)	Treatment in Judgment of 8 May 2025
Mental health crisis caused by the proceedings. Agitation and poor sleep.	Quoted at paragraph 21. Used to establish clinical evidence existed.
Advocacy support requested to ensure full engagement.	Quoted at paragraph 37. Used to conclude the applicant was not unfit to attend — only that she required support. This passage is then used to support the finding that she waived her right to be present.
Finding B: Difficulty processing evidence perceived as untrue — named as a specific, identified clinical feature of her presentation.	NOT QUOTED. NOT ACKNOWLEDGED. NOT ADDRESSED ANYWHERE IN THE JUDGMENT.
Finding C: Autism assessment pending. May influence how she perceives and processes information in the proceedings.	NOT QUOTED. NOT ACKNOWLEDGED. NOT ADDRESSED ANYWHERE IN THE JUDGMENT.

2.2 How the Judgment Characterised the Applicant

Having received Dr Poyser's letter, the final hearing attendance notes, the applicant's Section 25 statement, and the 6 May 2025 email — MacDonald J characterised the applicant in the judgment of 8 May 2025 as follows:

Para	Characterisation	Source
39	<i>"...the wife's very late application to adjourn this hearing stands at the end of a very long line of what I am satisfied is deliberately obstructive behaviour, designed to hinder the implementation of the final order..."</i>	Judgment of 8 May 2025, paragraph 39
38	The judgment used the applicant's continued correspondence during her documented clinical crisis as evidence of her capacity to attend: <i>"the wife has been in regular correspondence with the court regarding this litigation and has issued further applications."</i> This correspondence — produced during a period Dr Poyser described as a mental health crisis — is cited as evidence against the very crisis Dr Poyser described.	Judgment of 8 May 2025, paragraph 38
43	<i>"Whilst often verbose and rambling, the wife's arguments are tolerably clear from those documents... Her position would not be materially improved by adjourning this matter to permit her attendance."</i>	Judgment of 8 May 2025, paragraph 43
68	An indemnity costs order was made against the applicant. The judgment characterises her conduct as <i>"persistent poor litigation conduct."</i> No reference is made to the clinical evidence. No reference is made to Part 3A. No reference is made to Dr Poyser's letter except the one sentence that supported the finding she could attend.	Judgment of 8 May 2025, paragraph 68

2.3 What the Judgment of 18 April 2024 Did with the Witness Box Evidence

The final judgment of 18 April 2024 — the judgment MacDonald J confirmed he had read everything in order to produce — contains this finding at paragraph 64:

> "The wife's presentation in the witness box on the issue of alleged non-disclosure bordered on the concerning. Her belief that the husband has vast hidden assets is so powerfully held, and such was her urgent need to impress that belief on the court from the witness box, that at times everything she wished to say on the matter tried to emerge at once, causing her very obvious distress... I am satisfied she has become obsessed with demonstrating that in the context of this litigation."

Source: MacDonald J, judgment of 18 April 2024, paragraph 64.

The judgment of 18 April 2024 contains 111 paragraphs. It contains zero references to the applicant's PTSD. It contains zero references to her autism. It contains zero references to the participation measures MacDonald J had himself ordered. The clinical explanation for the presentation described at paragraph 64 — present in oral evidence, in counsel submissions, and in the medical record the judge confirmed he had read — does not appear anywhere in the judgment.

Dr Poyser's letter, written six weeks after this judgment, identified the specific clinical phenomenon that paragraph 64 describes — and attributed it to autism and the difficulty processing evidence perceived as untrue. Dr Poyser's letter was filed with the court on 8 May 2025. Findings B and C of that letter were not quoted in the judgment of 8 May 2025.

2.4 Notice Filed with the Court of Appeal Before the Hearing — and the Hearing That Proceeded

The applicant filed her 6 May 2025 email with the Court of Appeal at 17:49 on 6 May 2025. It stated, in terms, that she did not feel safe with the judge, that the cost order had been made against her autism and PTSD symptoms, and that the findings about her witness box presentation were unsupported by the oral evidence. The transcript of oral evidence was attached.

The Court of Appeal received the email. It did not respond before the hearing. The hearing on 8 May 2025 proceeded. MacDonald J delivered judgment. The applicant was not present. The adjournment application she had filed at 12:41am that morning was refused.

The recusal application the applicant had been filing since 10 February 2025 — almost daily, by her account — had not been determined by any court. It was not determined before the hearing on 8 May 2025. It was not determined after it. The first and only institutional response came on 10 June 2025 — 33 days after the first urgent email — when an administrative officer of the Court of Appeal informed the applicant that her application should have been filed in the High Court, not the Court of Appeal. No determination was ever made.

PART 3 — WHAT HAPPENED NEXT

The following sequence is established by independent third-party clinical records. None of the documents in this Part were produced by the applicant. None were produced by anyone with a connection to the Family Division proceedings.

3.1 CLAAS — Central London Adult Autism Service — 17 June 2025

On 17 June 2025 at 10:51, Sophia Iacovou (Assistant Psychologist, supervised by Dr Catherine Cheung, Consultant Psychologist), Central London Adult Autism Service, Central London Community Healthcare NHS Trust, sent a follow-up email to the applicant summarising a telephone discussion held that morning. The email was copied to the applicant's GP surgery (Stanhope Mews Surgery, SW7). It records the following:

Item	Content
Court failure documented by CLAAS	<i>"You shared with us that you have been struggling to navigate the case recently as you feel that your needs have been ignored by the court. You shared that you have not been provided with the necessary support to participate in the case or advocate for your needs."</i>
Clinical distress documented	<i>"The distress that this has caused is affecting your sleep and mood. You shared that you have been grinding your teeth, having trouble sleeping, and feeling more agitated than usual."</i>
Post-diagnostic support scheduled	<i>"We have organised three virtual sessions of post-diagnostic support, to begin on Thursday 19th June."</i> — Three days after this email was sent, the autism service raised emergency concerns about the applicant's mental state. The first session never took place.
Letter of support committed	<i>"We will be in contact with you within the next 1-2 weeks with a letter of support, outlining your circumstances and needs, as well as our recommendations for reasonable adjustments to be made by the court."</i>

Source: Email, Sophia Iacovou, CLAAS / CLCH NHS Trust, 17 June 2025 10:51, to Nadia Zahmoul, CC: Dr Catherine Cheung and Stanhope Mews Surgery GP.

3.2 The Applicant's Account — 17-20 June 2025

The following is the applicant's direct account of the 48-hour period. It is available as a signed statement to the police on request.

> When I received the judgment I went into shock. I could not speak. I was mute for approximately 48 hours. I was covered in bruises — self-inflicted. There was blood. My daughter found me and called an ambulance and the police. I felt trapped. I felt powerless.

I collapsed. The judgment was so wrong — so completely at odds with what had actually happened — that I could not process it. I could not function. The CLAAS team knew how severe the impact was. They were very concerned.

Direct testimony — Nadia Zahmoul, 24 March 2026. Available as a signed statement on request.

3.3 Chelsea and Westminster Hospital — Emergency Department — 20 June 2025

The following facts are drawn from the verified Emergency Department Departure Summary, Chelsea and Westminster Hospital NHS Foundation Trust, MRN 80718960. The document was produced by clinicians with no connection to the Family Division proceedings.

Clinical Fact	Detail from the Clinical Record	Source
Date and time	20 June 2025 at 06:09	ED Depart Summary (Verified)
Source of referral	Ambulance (BIBA — brought in by ambulance)	Initial Assessment
Clinical presentation on arrival	Unusual behaviour, erratic. Disorganised speech, movement on arrival. Attempted to abscond.	Initial Assessment
Preoccupation documented by LAS	<i>"She also appeared to be fixed on the courts and kept asking LAS if they were sent by them."</i>	Brief Summary
Collateral history from daughter	<i>"Her daughter reported a decline in Nadia's behaviour over the past 48 hours."</i>	Brief Summary
Autism service — prior day	<i>"Currently open to the autism service... Concerns were raised about Nadia's mental state by the autism service on 19/06/25 and she was referred to CNWL SPA for triage."</i>	Impression
Medication administered	Droperidol 10mg IM (06:23); Lorazepam 2mg oral (12:11); Paracetamol 1g oral	Medications Administered
Alcohol and substances	Ethanol: less than 100mg/L. Not detected.	Blood Sciences
Clinical impression	Significant deterioration and episode of agitation. Referred to SK&C HTT for further assessment and containment.	Impression

3.4 The Timeline

Date	Event	Source
8 May 2025	Judgment delivered. Applicant absent. Adjournment refused. All applications dismissed totally without merit. Indemnity costs ordered. No reference to disability in the judgment.	MacDonald J judgment (approved transcript)
17 June 2025 10:51	CLAAS documents distress, court failures, and clinical concerns in writing. Post-diagnostic sessions scheduled for 19 June. Letter of support with reasonable adjustments committed. CC: GP.	CLAAS email — Sophia Iacovou / Dr Catherine Cheung
19 June 2025	Autism service raises concerns about the applicant's mental state. Referred to CNWL SPA for triage. Post-diagnostic session was scheduled for this date.	A&E notes (internal clinical reference)
20 June 2025 06:09	Applicant brought to A&E by ambulance. Droperidol 10mg IM administered. Clinical record documents preoccupation with the courts. Daughter reports 48-hour decline. No alcohol or substances detected.	Chelsea and Westminster Hospital ED Depart Summary (Verified)

PART 4 — THE RECUSAL APPLICATION: A COMPLETE RECORD

The following table sets out the complete record of the recusal application. Every entry derives from a primary document. The application was filed from 10 February 2025. It was received by the Court of Appeal. It was never determined by any court at any level. The judge against whom it was directed continued to sit throughout.

Ref	Date	Action	Institution	Response
R-01	10 Feb 2025	First N244 recusal application filed. Applications filed almost daily thereafter.	Court of Appeal	None
R-02	13 May 2025	Formal N244 recusal application signed and filed. Grounds: judicial bias; disability	Court of Appeal	None at this stage

		discrimination; suppression of clinical evidence; defamatory findings about witness box presentation.		
R-03	6 May 2025 17:49	Urgent email: application must be sealed and 8 May hearing stayed. Applicant states she does not feel safe with the judge. States cost order was made against her autism and PTSD symptoms. Transcript of oral evidence attached.	Court of Appeal (CMSB + Urgent Work)	No response before hearing
R-04	7 May 2025 09:37	Chase to Mr Singh. Application must be sealed urgently.	Court of Appeal	No response before hearing
R-05	8 May 2025 03:46	Email sent at 3:46am to Mr Maher. States judge <i>"intentionally suppressed my mental health disabilities and attributed my presentation in the witness box to bad character."</i> Requests appointment of different judge.	Court of Appeal	No response before hearing
R-06	8 May 2025	HEARING PROCEEDS in applicant's absence. Judgment delivered. All applications dismissed TWM. Indemnity costs ordered.	MacDonald J — Family Division	N/A — hearing proceeds
R-07	12–13 May 2025	Escalation to Parliament (Lord Chancellor, Prime	Parliament, AG, Master of the Rolls	No substantive response from any

		Minister, Lord Speaker, Margaret Hodge MP). Formal recusal application sent to Master of the Rolls, Attorney General (named), Anti-Corruption Champion, Human Rights Watch, 100+ MPs and Lords.		recipient
R-08	10 Jun 2025 15:27	ONLY INSTITUTIONAL RESPONSE RECEIVED. Peter Cobourn (Administrative Officer, Court of Appeal): <i>"Your application notice dated 13 May 2025 seeking the recusal of MacDonald J should be made in the High Court, not the Court of Appeal."</i>	Court of Appeal (Peter Cobourn, Administrative Officer)	33 days after first urgent email. Redirection only. No determination.
R-09	17 Jun 2025	CLAAS documents distress and court failures. Autism service raises concerns 19 June.	NHS (CLAAS / CLCH)	Clinical referral only
R-10	20 Jun 2025 06:09	Applicant admitted to Chelsea and Westminster A&E by ambulance. Droperidol 10mg IM. Fixed on the courts (A&E notes). 48-hour decline reported by daughter.	Chelsea and Westminster Hospital NHS	Acute psychiatric treatment
R-11	17 Jun – 30 Jul 2025	Further recusal applications sent to 100+ recipients including Family High Court registry, Court of Appeal Urgent Work, Attorney	Multiple institutions	No determination by any court

		General (named individuals), Master of the Rolls.		
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The hearing that proceeded (R-06) and the acute psychiatric crisis (R-09, R-10) are flagged. The only institutional response received (R-08) was administrative redirection. No judicial determination was ever made by any court at any level.

PART 5 — DOCUMENTS AVAILABLE FOR PRODUCTION ON REQUEST

Ref	Document	Date	Description
DOC-01	Dr Leigh Poyser MBBS MRCPsych — Letter to Court	26 Mar 2025	Consultant Psychiatrist, South K&C CRHTT, CNWL NHS. Four findings: crisis; difficulty processing evidence perceived as untrue; autism assessment pending; advocacy support requested. Dr Poyser has been shown the relevant passages of the judgment. Her response is available as a statement to police on request.
DOC-02	Gloucestershire CRHTT Assessment	4 Mar 2025	James Pullen, Trainee Advanced Clinical Practitioner, Gloucestershire Health and Care NHS. Crisis assessment. PTSD confirmed. Acute stress reaction. Prior A&E admission with haloperidol administered.
DOC-03	CLAAS — Summary of telephone discussion	17 Jun 2025	Sophia Iacovou, Assistant Psychologist / Dr Catherine Cheung, Consultant Psychologist, Central London Community Healthcare NHS Trust. Distress documented. Court needs ignored. Post-diagnostic sessions scheduled from 19 June.

			Letter of support with reasonable adjustments committed. CC: GP.
DOC-04	MacDonald J — Judgment (approved transcript)	8 May 2025	17 pages, 68 paragraphs. Paragraphs 21-22 and 37-38 quote Dr Poyser's letter selectively. Paragraphs 39-44 characterise non-attendance as deliberately obstructive behaviour. Indemnity costs ordered at paragraph 68.
DOC-05	MacDonald J — Judgment (18 April 2024, approved)	18 Apr 2024	111 paragraphs. Zero references to disability or Part 3A. Paragraph 64 characterises witness box presentation as obsession. The clinical explanation for that presentation was present in the bundle the judge confirmed he had read.
DOC-06	MacDonald J — Sealed Order	8 May 2025 (sealed 5 Jun 2025)	All applications dismissed totally without merit. Court signs transfer documents under s.39 Senior Courts Act 1981. Indemnity costs ordered.
DOC-07	Final Hearing Attendance Notes	Feb-Mar 2024	Pupil notes of the final hearing. Records judge's direct statement to applicant confirming he had read everything in the bundle (" <i>I know</i> "). Records judge's own question " <i>Did you say Nadia had PTSD?</i> " Records AC KC's words " <i>I have no objection to NZ being supported if she needs that.</i> " Official transcript applied for under Form EX107 — outstanding.
DOC-08	Email: CA-2024-001342-A — FOR YOUR URGENT ATTENTION	6 May 2025	Filed with Court of Appeal two days before the hearing. States: cost

			order made against autism and PTSD symptoms; findings about witness box presentation unsupported by oral evidence; applicant does not feel safe. Transcript of oral evidence attached. Received by Court of Appeal. No response before hearing.
DOC-09	N244 Recusal Application (signed) and supporting affidavit	13 May 2025	Formal recusal application. Grounds: judicial bias; disability discrimination; defamatory findings; cost order against PTSD and autism symptoms.
DOC-10	Master Meacher — Court of Appeal redirection	10 Jun 2025	" <i>Application dated 13 May 2025 seeking recusal of MacDonald J should be made in the High Court, not the Court of Appeal.</i> " Received 33 days after first urgent email. No determination made.
DOC-11	Chelsea and Westminster Hospital — ED Department Summary (Verified)	20 Jun 2025	MRN 80718960. Admission by ambulance 06:09. Droperidol 10mg IM. Preoccupation with courts documented. Autism service concerns raised 19 June. 48-hour decline reported by daughter. No substances detected. Referred to SK&C HTT.
DOC-12	Direct testimony of the applicant	24 Mar 2026	Account of the 48-hour crisis: mute; self-inflicted injuries; blood discovered by daughter who called ambulance and police; felt trapped, powerless, collapsed. Available as a signed statement on request.
DOC-13	Recusal correspondence (full)	May-Jul 2025	Complete email chain from 6 May 2025 to 30

			July 2025. Available on request.
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This referral is submitted as part of a criminal referral package addressed to: Metropolitan Police Service — Serious and Organised Crime Command; Crown Prosecution Service — Special Crime Division; National Crime Agency — Economic Crime Command. It may be considered as a self-contained referral independently of that package.